

Swelling location localises the problem

Lacrimal gland inflammation is superotemporal; lacrimal sac infection is inferomedial.

A watery eye with swelling

Site / finding	Likely process
Superotemporal upper lid; S-shaped ptosis	Dacryoadenitis: lacrimal gland inflammation.
Below medial canthus; tender and red	Dacryocystitis: infected lacrimal sac.
Purulent reflux through a punctum on sac pressure	Supports lacrimal sac infection with obstructed drainage.
Everted punctum without a sac lump	Ectropion impairs entry of tears into the drainage system.

Recognise infection rather than simple watering

Ask about pain, fever, the duration of epiphora and previous drainage problems. A painful lacrimal sac swelling is different from uncomplicated congenital obstruction. Significant swelling or spread beyond the sac needs prompt assessment, particularly if eye movements or vision become abnormal. The location of the primary swelling is a useful first discriminating clue.

Infant epiphora: drainage or another eye problem?

A clear cornea and absent photophobia support uncomplicated drainage obstruction, but warning signs need reassessment.

Clinical pathways

What accompanies the watering?

Clear cornea; early-life watering / sticky discharge

Consider congenital nasolacrimal obstruction; teach sac massage and review.

Delayed fluorescein dye disappearance

Supports impaired drainage; interpret with the examination.

Painful medial swelling or fever

Assess for dacryocystitis.

Photophobia, corneal haze or enlargement

Assess for congenital glaucoma or another significant ocular cause.

Use the test for its purpose

Dye disappearance measures how effectively tears leave the ocular surface. Retained dye supports impaired drainage but does not itself diagnose infection or locate every possible obstruction. Many uncomplicated infant obstructions improve with conservative care; persistent or atypical symptoms require follow-up rather than an assumption that all watering will resolve.